



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : عفاف احمد السيد عيسوى
المحافظة : القليوبية الحي / المدينة : بها
رقم البطاقة : 27206260102469 رقم التليفون : 01281289063
العمر (تاريخ الميلاد) : 26-06-1972 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقيم بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة :
الاسم : د/ندى حسن الوظيفة :
الاسم : د/رمين رمضان الوظيفة :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المدونة في هذا النموذج صحيحة.
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **عبد الله أحمد عيسى**
National ID: **27206260102469**



تقرير طبي

المرضى

تحتية طبية وبعد

بشأن تقرير طبي للمريض

إهداء للطلب المقدم من

رقم طبي:

رقم:

تتأكد بأن توضح لهذا التقرير عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة التفتيش :
PM 01:17 05-07-2026

وبعد التفتيش والموصات والمتابعة وجدته

المرضى يعاني من عدم قدرة على الحركة و يحتاج الى Supportive and palliative Treatment

توقيع :

دكتور محمد عبد

التاريخ :

05-07-2026

الطبيب المعالج :
الأخصائي الدكتور

جمهورية مصر العربية
بطاقة تحليل الشخصية

عظاف
أحمد السيد محمود
شركة الكهرباء - شقة الكهنة
مركز مستشفى - بني سويف

٢٠٢٢/٠٩/٢٦ ٠١/٠٢/٢٠٢٢

KE5654972

٢٠٢٢/٠٩/٢٦ ٠١/٠٢/٢٠٢٢

دبلوم تجارة

النسب
التي
عصام أحمد محمود حسن
البطاقة مارية حسن

٢٠٢٢/٠٩/٢٦ ٠١/٠٢/٢٠٢٢

7-8-2025

MDT

Patient Name	عفاف احمد السيد	Age	52
Sex	Female	Special habits	
Family History		Co-morbidity	
Surgical History		Telephone	01281289063
Menopausal Status	POSTPREMENOPAUSAL	Marital Status	
MEDICAL ID	108/2025	ID	27206260102469

Pathology Bx: IDC GII

ER:0

PR:0

Her:3

PATIENT HX Metastatic left breast cancer to bone, liver and LNs
Received Taxol, Herceptin x20 weeks
21/5/2025: Increase in size of one of the right axillary lymph nodes for
Sonography-> done on 21/7 -> Few enlarged axillary LNs at level I, the largest
one is measuring 18x7mm
CT.CAP 21/7/2025-> RD and multiple lesions, stationary course of the disease

MDT outcome and plan Consider secondline Xeloda + Tykerb (31/7/2025)

MDT DECISIONCONTINUE FOR 9WKS TAXOL+HERCEPTINX3 CYCLE
NEW CT CAP WITH COMPARSION CT 7\2025

Signature

Signature

د. الزهراني مصطفى
21/7/2025
C. ع. ع. ع.

Patient's Name: AFAF AHMED ALSAYED
Date: 2026-03-30
Referred by: Prof.Dr. CAP+C
Hospital: Gustave Roussey International Egypt

**MULTISLICE POST CONTRAST CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS:**

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

In reviewing the previous study report dated 4-10-2025

(I) Chest:

- Still noted bilateral mosaic attenuation of both lungs
- No sizable parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardioemagly
- Still noted left breast UOQ lesion and thickened skin ,

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density with no hepatic focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys ,apart from left renal cortical cyst, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulac.

Patient's Name: AFAF AHMED ALSAYED
Date: 2026-03-30
Referred by: Prof.Dr. CAP+C
Hospital: Goustave Roussey International Egypt

- Small fat/omentum containing right inguinal hernia is noted.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- L3 small sclerotic lesion is still noted.
- Dorsolumbar spondylotic changes are noted.

OPINION:

- **Follow up status showing rather stationary course since last study**

Dr.Nourhan Ewis

Dr.Nourhan Ewis

المرضى	الطلب رقم	Request No	: 13003628
72062-101024	الطلب رقم	Request Date	: 13/01/2025
318-102-25	التاريخ	Verification Date	: 21/01/2025
Female	التاريخ	Financial Category	: Out Patient
62 Y	التاريخ	Department	: Mrs. Mayada Hegazy
Remitting clinic:		Registered by	

Pathology Report

Diagnosis: Left breast cancer.
 Request of specimen: Slide revision

Examination:
 One paraffin block labeled by (P012-25) and one H&E slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Lymphovascular invasion: Could not be detected within the limits of biopsy received

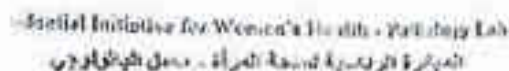
Diagnosis:

Left breast mass, Slide revision:
 Invasive duct carcinoma, Grade II with focal DCIS.
 For further IPT (ER, PR, Her2 and Ki67).

Signature
 Prof. Dr. Lubna Omar
 Dr. Menna Nabil

Verified by:
 Dr. Heba Hasan





Patient Name	الإسلام / عفاف أحمد - السيد حوسري	Request No.	: 13003629
Patient No.	: 27206260102459	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 20/01/2025
Sex	: Female	Financial Category	: فورام دار السلام - فئة طلقة
Age	: 2 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs. Mayada Hozazy

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

* Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

- IFT (Positive control showed appropriate reactivity);
- ER: No nuclear reaction in tumor cells (Allred score 0/3).
- PR: No nuclear reaction in tumor cells (Allred score 0/3).
- Her2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.
IP1:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- K67: Positive in 10% of tumor cells.

* Spring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, $\geq 1\%$; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr. Toka Medhat

Printing Date : 30/01/2025 11:15:04

Page 1 of 2

Printed By : Mrs. Mayada Honaizy



خريطة رقم (4) - الميناء القديم - مصر القديمة - ١٩٥٧ -
Recon No. (4) - First Floor-Building No. 907- Nile Corniche - Mitre El Qadima - Cairo, Egypt

Scanned with
CamScanner

Patient Name: الأستاذة / د. هبة السيد حسني
Patient No.: 27208260102469
Path No.: SR-105/25
Sex: Female
Age: 52 Y
Referring Doc:

Request No.: 13003629
Request Date: 13/01/2025
Verification Date: 29/01/2025
Financial Category: برنامج دول شلوم - فئة خولة
Department: Out Patient

Registered by: Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Gamal Abdelaziz
Dr. Heba Hassan

Verified by:
Dr. Toka Medhat

Printing Date: 30/01/2025 11:15:04

Page 2 of 2

Printed By: Mrs. Mayada Hegazy



مركز أمراض النساء - مبنى (4) - الدور الأرضي - مبنى (987) - دور الأرضي - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987, Nile Corniche - Mar El Qasr - Cairo, Egypt

Patient name: *Yfar Ahmed*
Date: *4/1/2025*
Prof. Dr. *Samar Khamis*

US Examination Of Both Breasts Revealed:

- A.R.: Mixed subo-fatty-glandular breast parenchyma.
- A well-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped heterogeneous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm, associated mild edematous changes manifested by parenchymal coarsening and skin thickening are noted.
- The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non-specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.

Best regards,
Dr. Esraa Hatem

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BIRADS 0:* Needs further investigations.
- BIRADS 1:* Normal findings.
- BIRADS 2:* Benign.
- BIRADS 3:* Probably benign.
- BIRADS 4:* Probably malignant.
- BIRADS 5:* Malignant.
- BIRADS 6:* Proven pathologically malignant.

Dr. Esraa Hatem
Radiology specialist

المعهد القومي للأورام
National Institute of Oncology

د.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د.د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Specimen
Data

Patient Name: د.د/ عبلة سيد محمود عبد العليم

Age: 51 y

Gender: Female

Referred By Prof. Dr. أسعد البحري

Nature of the specimen: True-Cut Biopsy

Specimen Diagnosis: Left Breast Mass

Date of Recd: 5-1-2025

Code Number: P0012-15

Report Issued At: 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

4 (1-2) grayish white firm cores of average length 1 cm, totally submitted.

Findings:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild periductal lymphocytosis.
- No intraductal components were detected.

Diagnosis:

Left Breast Mass, True-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade II

Immunophenotype: (ER, PR, HER2, NEU & Ki67)



Eman A. Abd Elmaoged

Abla S. Mahmoud

من الإبراهيمية أمام معرض شهاب مصر - بني سويف
01017211026/ت



Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

> Regressive course of the:

1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.

- > **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- > Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- > No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- > **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- > No evidence of pleural effusion, pleural thickening or masses seen.
- > Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- > Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- > The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- > No intra-hepatic or extra-hepatic biliary radicles dilatation.
- > Normal size & parenchymal texture of the spleen with no focal lesions seen.
- > Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- > Normal appearance of the pancreas and both suprarenals.
- > Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- > Normal appearance of the other pelvic organs.
- > No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- Increase in size of one of the right axillary lymph nodes for sonographic assessment.
- Overall regressive course as described.

Dr. Mohamed Mohsen, M.D.

جوسناف
رئيس
الاستشارة



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

وزارة الصحة
الجمهورية العربية السورية

30/12/2026

0

MDT FORM

Name:	Alraf Ahmed El-Sayed	MRN:	648
Age:	54 Y	ID:	27206260107469
Menopausal Sta:		Phone:	01062013901
NRS SCORE:		Marital Status:	
BHT:		Family History:	
Gender:		Reason OF MDT:	
CO Morbidity:		Progressive disease:	
Surgical History:		ECOG Performance Status:	
		Q10 1 2 3 4	

Clinical Data:

BX:	IOC
ER: 0/8	PR: 0/8
HER2: 3+	KI67: 10%

History:

MBC to bones and lymph nodes
Received Paclitaxel - Herceptin
Then Tykerb - Xeloda
Stereotactography:

Biomarker/Molecular:

Met Workup:

Complaining from headache and slurred speech
C/E oral moniliasis

Brain MRI without contrast: Multiple SOL.

CT CAP 15/8: SD of LP sclerotic lesion

Bone Scan 20/8: No mets seen

To start Docicicab, Methotrexate

New Brain MRI + C

WCI

Recommendations:

MDT Outcomes & Plan:

New Brain MRI + C

WCI

Refer for TCM-1 (NGO)

For Gemtastine - Carboplatin meanwhile



T N M

Right Side

Left Side

Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Dr. [Signature]	Dr. [Signature]		

Patient Name : علي احمد السيد
Patient Info. : Female 53Y
Department : Out Clinic
Profile NO. : 108-2025

Patient Id : 3474
VisitId : 5213
Collection Date : 04/12/2025 11:39:29
Reporting Date : 04/12/2025 11:44:10

COMPLETE BLOOD PICTURE

Test	Result	Unit	Reference Range
CBC			
Hemoglobin (Hb)	13.2	g/dL	12 - 16
RBCs	4.61	$10^6/\text{cmm}$	3.8 - 4.8
Haematocrit (HCT)	39.6	%	36 - 46
MCV	85.9	fL	76 - 95
MCH	28.6	pg	26 - 32
MCHC	33.3	g/dL	32 - 36
RDW	15.80	%	11.6 - 14.5
Platelet count	216	$10^3/\text{cmm}$	150 - 450
WBCs	L 3.44	$10^3/\text{cmm}$	4 - 10
Differential white cell Count			
	Relative (%)	R.Range	Absolute ($\times 10^3/\text{cmm}$)
Neutrophils	L 39.50	40 - 70	L 1.36
Lymphocytes	42.50	20 - 45	1.46
Monocytes	6.10	2 - 8	0.21
Eosinophils	H 11.70	1 - 6	0.40
Basophils	0.20	0 - 1	0.01

Signature
Dr. Sahar Mansour

MDT FORM

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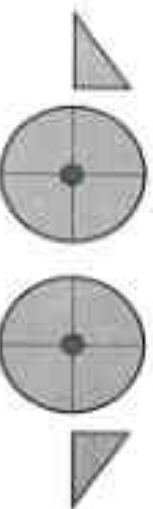
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Name	مينا	MRN	1081225
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC G1			R	L
ER:	PR:	HER 2:	Ki67:		
018	018	+3			

Sonomammography:



Mets Workup:

TNM

Mets breast cancer to Vxv & LN
 received 1st line tx with cyclophosphamide + Herceptin
 best CT CAP → SD. (Copy and to histology)

Recommendations:

Per MDT:

i.e. AD Copy and to

12/12/2021. Consider 2nd line Tx with
 Docil

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

7-8-2025

elevation
private equity

MDT

Patient Name	الاسم الكامل	Age	Special habits Co-morbidity Telephone	52
Sex	Female			
Family History				
Surgical History				
Menopausal Status	POSTMENOPAUSAL	Marital Status		
MEDICAL ID	108/2025	ID		27206260102469

Pathology Bx: IDC GII

ER:O

PR:O

Her:3

PATIENT HX

Metastatic left breast cancer to bone, liver and LNs
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Sonography-> done on 21/7 -> Few enlarged axillary LNs at level I, the largest
one is measuring 18x7mm
CT,CAP 21/7/2025-> RD and multiple lesions, stationary course of the disease

MDT outcome
and plan

Consider secondline Xeloda + Tykerb (31/7/2025)

MDT DECISION

CONTINUE FOR 9WKS TAXOL+HERCEPTIN X3 CYCLE
NEW CT CAP WITH COMPARISON CT 7/2025

signature

Signature
15/08/2025
15/08/2025

Signature

Signature

Patient Name : علي احمد السيد
Patient Info. : Female 53Y
Department : Out Clinic
Profile NO. : 108-2025

Patient Id : 3474
VisitId : 5213
Collection Date : 04/12/2025 11:39:29
Reporting Date : 04/12/2025 11:44:10

COMPLETE BLOOD PICTURE

Test	Result	Unit	Reference Range
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RDW	15.80	%	11.6 - 14.5
Platelet count	216	$10^3/\text{cmm}$	150 - 450
WBCs	L 3.44	$10^3/\text{cmm}$	4 - 10
Differential white cell Count			
	Relative (%)	R.Range	Absolute ($\times 10^3/\text{cmm}$)
Neutrophils	L 39.50	40 - 70	L 1.36
Lymphocytes	42.50	20 - 45	1.46
Monocytes	6.10	2 - 8	0.21
Eosinophils	H 11.70	1 - 6	0.40
Basophils	0.20	0 - 1	0.01

Signature
Dr. Sahar Mansour

MDT FORM

/ / 20

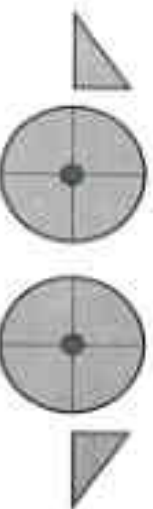
G

Name	مينا	MRN	1081225
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC G1			R	L
ER:	PR:	HER 2:	Ki67:		
018	018	+3			

Sonomammography:



Mets Workup:

TNM

Mets breast cancer to Vxv & LN
 received 1st line tx with cyclophosphamide + Herceptin
 best CT CAP → SD. (Copy and to histology)

Recommendations:

Per MDT:

i.e. AD Copy and to

12/12/2021. Consider 2nd line Tx with
 Docil

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name : الأستاذة / عفاف أحمد السيد عيسى
Patient No. : 27206260102469
Path No. : SR-105/25
Sex : Female
Age : 52 Y
Referring Doc :

Request No. : 13003629
Request Date : 13/01/2025
Verification Date : 21/01/2025
Financial Category : فئة المرضى - Out Patient
Department :
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labeled by (P012-25) and one H&E slide were received.

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- DCIS: Focally positive.
- Angiolymphatic invasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature

Prof. Dr. Lubna Omar
Dr. Menna Nabli



ERM036748357

Verified by :
Dr. Heba Hasan

Printing Date : 22/01/2025 08:24:23

Page 1 of 1

Printing By : Internal



7-8-2025

elevation
private equity

MDT

Patient Name	الاسم الكامل	Age	Special habits Co-morbidity Telephone	52
Sex	Female			
Family History				
Surgical History				
Menopausal Status	POSTMENOPAUSAL	Marital Status	01261269063	
MEDICAL ID	108/2025	ID	27206260102469	

Pathology Bx: IDC GII

ER:O

PR:O

Her:3

PATIENTHX

Metastatic left breast cancer to bone, liver and LNs
Received Taxol, Herceptin x20 weeks
21/5/2025: Increase in size of one of the right axillary lymph nodes for
Sonography-> done on 21/7 -> Few enlarged axillary LNs at level I, the largest
one is measuring 18x7mm
CT,CAP 21/7/2025-> RD and multiple lesions, stationary course of the disease

MDT outcome
and plan

Consider secondline Xeloda + Tykerb (31/7/2025)

MDT DISCUSSION

CONTINUE FOR 9WKS TAXOL+HERCEPTINX3 CYCLE
NEW CT CAP WITH COMPARISON CT 7/2025

signature

15/08/2025
15/08/2025





Patient Name : الأستاذ / عفاف احمد السيد عيسوي	Request No. : 13003629
Patient No. : 27206260102469	Request Date : 13/01/2025
Path No. : SR-105/25	Verification Date : 21/01/2025
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil



Verified by :
Dr.Heba Hasan



IEYA SCAN

NAME : Afaf Ahmad

DATE: 8/1/2025

PROF .DR; SaedAlbehery



مركز
هي للأشعة

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- ACR B breast parenchyma
- An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen
- Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification
- No dilated lactiferous ducts
- Normal right skin thickness and contour.
- Normal nipple areola complex.
- Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila

OPINION:

- Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)

Much obliged,
DR .Eman AbdelRahman ,MD

001119598889 - 01005425312 @ 0822167074

بنى سويف - ش احمد عرابى برج زيادة 2 - أمام قصر الثقافة

بنى سويف - أمام أجيال للملابس



Patient Name: AFAN AHMED ALDAVEED

Patient ID: 324-154

Gender:

Nuclear Imaging Endorsment

Study Name: BONE SCAN

Date & Time: 2/3/2015

Manufacturer Model: VARIUM



فيجنا لاب

لتحاليل الخلايا والأنسجة والأورام

د.د/ إيمان أحمد عبد المونود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د.د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. د.د/ سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At : 5-1-2025

Code Number: P0012-25

Report issued A : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 N.E. & Ki67).



Eman A. Abd Elmaaged

Abla S. Mahmoud

ش. الإبراهيمية امام معرض شباب مصر - بني سويف

ت/ 01017211026

Pathology Report

Patient name: Vis. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03,2021

Tc^{99m}-M.DP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- No scintigraphic evidence of bone deposits throughout the scanned skeleton.

Mahmoud Saban Abd Al-Alim, MD, EBIM

European Board of Nuclear Medicine

فيديا لاب

لتحاليل الخلايا والأنسجة والأورام

د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. أ.د. سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At: 5-1-2025

Code Number: P0012-25

Report issued A: 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests. Individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis.
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass: Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER2/2 NLU & Ki67).



Eman A. Abd Elmaoged

Abla S. Mahmoud

ش. الأبراهيمية امام معرض شفايف مصر - بني سويف

ت/ 01017211026

One Size
FUTURE ROSE SCALE

Patient name: Afaf Ahmed

Date: 4/1/2025

Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped heterogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mild edematous changes manifested by parenchymal coarsening and skin thickening are noted.
- The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.

*Best regards,
Dr. Esraa Hatem*

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0:	Needs further investigations.
BIRADS 1:	Normal findings.
BIRADS 2:	Benign.
BIRADS 3:	Probably benign.
BIRADS 4:	Probably malignant.
BIRADS 5:	Malignant.
BIRADS 6:	Proven pathologically malignant

Dr. Esraa Hatem
Radiology specialist



Patient Name: Afaf Ahmed Al Sayed El sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes; the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- **Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.**
- **Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.**



Patient Name: Afaf Ahmed Al Sayed E. sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Med cal ID: 108/2025	Pet. ID: 96847

- Bilobar hepatic focal lesions for triphasic CT assessment.
- L5 tiny lytic lesion.
- For correlation with the unavailable previous studies.

Dr. Mohamed Mohsen, M.D.



Patient Name	الأستاذة / عفاف احمد السيد حسوي	Request No.	13003628
Patient No.	272082/0102469	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	28/01/2025
Sex	Female	Financial Category	اورام نادر السلام - نفقة التوالة
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Gamal Abdelatty
Dr Haba Hassan

Verified by :
Dr. Toka Medhat

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Page 2 of 2

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غرفة رقم (4) - الطور الأول - مباني رقم (917) - اورشليم الشرق - مصر الجديدة - القاهرة 1 - مصر
Room No. (4) - First Floor-Building No. 917- Nile Corniche - N.H. Qadima - Cairo, Egypt



Patient Name	: الأستاذة / طيف احمد السيد عسوي	Request No.	: 13003629
Patient No.	: 27206280102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: برنامج ممر السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5iP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Page 1 of 2

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مركز (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mian El Qadima - Cairo, Egypt



Patient Name	: الأستاذ / عفاف احمد السيد عيسى	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: فورم دو السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labeled by (P012-25) and one H&I: slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angiolinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature

Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

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Page 1 of 1

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شقة رقم (4) - الدور الأول - مبنى رقم (387) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 387- Nile Corniche - Miss El Qadima - Cairo, Egypt



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البيانات الشخصية
بطاقة

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C. 50110A
Age	52	ID	52. 5754. 527
Menopausal status		Phone	0158150472
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NPV

Clinical Data

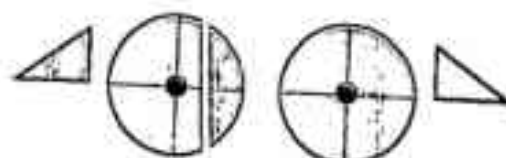
BX: ID 4

ER: 0 PR: 0 HER 2: +3 KI67:

Sonomammography:

Mets Workup:

R



TNM
- Mediastinal L-N
- Bilobar F. L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (paclitaxel) 30 (30)
Then Re-evaluate.

- baseline CT chest, PA etc. etc.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد العبد		د. محمد			

Name	كفان محمد سعيد محمد	MRN	
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC CT			R	L
ER:	PR:	HER 2:	KITV:		
0	0	+3			
sonomammography:					
Mets Workup:				TNM	
Metastatic to breast Ca to bone, liver, L. No Kecelated Taxol / Herceptin x 20 weeks CT. CAP 21/5/20 → PD of multiple lesion					

Recommendations:

For MDT for next plan of management

PD of axillary L.N.

21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : Axillary u/s	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- > Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up
- > Normal skin thickness and subcutaneous tissues
- > Normal vascular structure
- > Signature Dr . Nermien Ramadan
- > 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WIs
- Sagittal T1 W1
- Coronal T2 W1
- Axial D'WI and ADC
- Post contrast T1 series

MR findings:

- Well defined cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- No cerebral, cerebellar or brain stem areas of altered MR signal.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- Mild age matched brain involutional changes.

Dr.Zeyad El-Sayed,FEBR

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

➤ **Stationary course** of the:

1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.

➤ **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.

➤ Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.

➤ No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.

➤ **Stationary course** of the enlarged mediastinal lymph nodes being subcm.

➤ No evidence of pleural effusion, pleural thickening or masses seen.

➤ Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

➤ Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

➤ The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non-contrast study.

➤ No intra-hepatic or extra-hepatic biliary radicles dilatation.

➤ Normal size & parenchymal texture of the spleen with no focal lesions seen.

➤ Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

➤ Normal appearance of the pancreas and both suprarenals.

➤ Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

➤ Normal appearance of the other pelvic organs.

➤ No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

Date 22

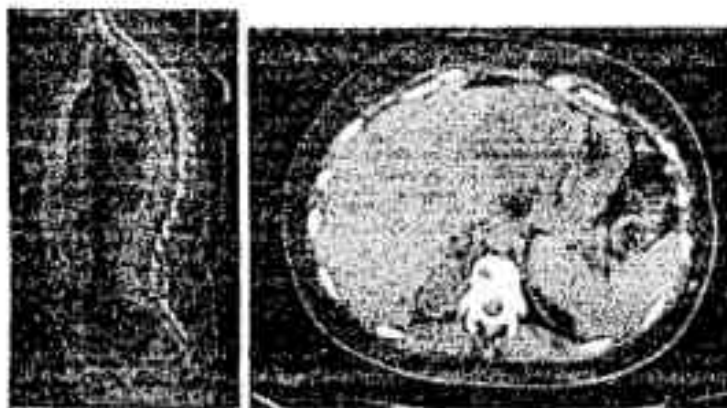
Dr. AYA GABR

Name : Afaf El Sayed

Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texture still showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentimetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.



Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
 1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
 2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

اسم المريض ثلاثي: ع. ل. م. / 10/19
رقم الملف: 1081722

Chronic pain clinic referral form

Mark the inclusion criteria : at least one

- # Persistent Pain Management Service (PPMS) input with patient **approval** to follow up for at least 6 months
- # Patient is cognitively capable, aware and agreeable to participate in the Pain management Program including self-management strategies and interdisciplinary teammembers as an option.
- **Worsening** post-surgical pain of more than 7 months duration (where a post-operative complication has been excluded and conventional analgesics max dose given)
- Newly diagnosed or suspected complex regional pain syndrome (CRPS) greater than 1 weeks after the triggering event
- Neuropathic pain (uncontrolled)
- Escalation of **symptoms** associated with escalation of **functional disability** (school attendance, engagement in physical activity, sleep, mood changes, frequent emergency department attendance)

Note the exclusion criteria :

- Ongoing untreated infection source
- Untreated addiction
- Unstable condition in need of inpatient care and monitoring
- Poorly controlled psychotherapy

Referral notes :

52 years old female patient with metastatic breast cancer to liver & lung on chemo & target (Herceptin) with R.C. but with **intractable pain**.
Please consider level one & assess regular pain therapy.

Referring Doctor's Name: C. L. M. Signature:

Date: 3/7/2022 Time: 11 am

Please consider
→ Anti-inflammatory dose
for ten days

C. 10/1/19

→ Neuropathic pain killer
Gabapentin 300mg twice daily

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847
1720	
1000	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST **PRIMARY REPORT**

- *Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:*



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted subcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afa ² Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1723 1000	Pat. ID: 96847

OPINION:

- *Stationary course of the disease process.*

Marina Essam, MSc



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السيد/الفضيلة
مختبة
مطر

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C 50, 1. A
Age	52	ID	20. 670. 1. 02
Menopausal status		Phone	01 58160475
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	new

BX:

Clinical Data

ER:

PR:

HER 2:

KI67:

Sonomammography:

Mets Workup:

Recommendations:

R

L

TNM
- Mediastinal L-N
- B. Lpar F. L

MDT OUTCOME & PLAN

1/2/2023 → for 1st line Taxol (herceptin) (IV) (SC)
then to evaluate.

- baseline CT chest, PA etc etc.

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

WPC DSHS-MDT 001
REVISED
MAY 10 2
Issue date: 1/9/2022

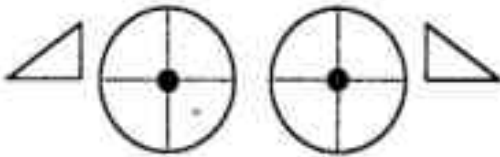
MDT FORM

/ / 20

G

Name	سافانة احمد	MRN	1081825
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC G1	R	L
ER: 0/8	PR: 0/8	HER 2: +3
Sonomammography:		
		
Mets Workup: Mets breast cancer to liver & LN received 1st line treatment weekly + Herceptin last CTCAP → SD. (Gy and to baseline)		
Recommendations: for MDT. 1.e. AP Gy and to baseline 5-2025 Consider 2nd line Xeloda-Tx Met		

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name	: الأستاذ / عفاف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil



Verified by :
Dr.Heba Hasan



IEYA SCAN

NAME : Afaf Ahmad

DATE: 8/1/2025

PROF .DR; SaedAlbehery



مركز
هي للأشعة

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- ACR B breast parenchyma
- An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen
- Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification
- No dilated lactiferous ducts
- Normal right skin thickness and contour.
- Normal nipple areola complex.
- Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila

OPINION:

- Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)

Much obliged,
DR .Eman AbdelRahman ,MD

001119598889 - 01005425312 @ 0822167074

بنى سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة

بنى سويف - أمام أجيال للملابس



Patient Name: AFAN AHMED ALDABEEN

Patient ID: 324-154

Gender:

Nuclear Imaging Endorsment

Study Name: BONE SCAN

Date & Time: 2/3/2015

Manufacturer Model: VARIUM



فيجنا لاب

لتحاليل الخلايا والأنسجة والأورام

د.د/ إيمان أحمد عبد المونود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د.د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. د.د/ سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At : 5-1-2025

Code Number: P0012-25

Report issued A : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 N.E. & Ki67).



Eman A. Abd Elmaaged

Abla S. Mahmoud

ش. الإبراهيمية امام معرض شباب مصر - بني سويف

ت/ 01017211026

Pathology Report

Patient name: Vis. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03,2021

Tc^{99m}-M.DP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- No scintigraphic evidence of bone deposits throughout the scanned skeleton.

Mahmoud Saban Abd Al-Alim, MD, EBIM

European Board of Nuclear Medicine

فيديا لاب

لتحاليل الخلايا والأنسجة والأورام

أ.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

أ.د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. أ.د. سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At: 5-1-2025

Code Number: P0012 -25

Report issued A : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis.
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass: Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER2/2 NLU & Ki67).



Eman A. Abd Elmaoqed

Abla S. Mahmoud

ش. الأبراهيمية امام معرض شفايف مصر - بني سويف

ت/ 01017211026

Pathology Report
[Signature Pathology Reports]

Patient name: Afaf Ahmed

Date: 4/1/2025

Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped heterogeneous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mild edematous changes manifested by parenchymal coarsening and skin thickening are noted.
- The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.

*Best regards,
Dr. Esraa Hatem*

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0:	Needs further investigations.
BIRADS 1:	Normal findings.
BIRADS 2:	Benign.
BIRADS 3:	Probably benign.
BIRADS 4:	Probably malignant.
BIRADS 5:	Malignant.
BIRADS 6:	Proven pathologically malignant

Dr. Esraa Hatem
Radiology specialist



Patient Name: Afaf Ahmed Al Sayed El sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes; the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.
- Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.



Patient Name: Afaf Ahmed Al Sayed E. sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Med cal ID: 108/2025	Pet. ID: 96847

- Bilobar hepatic focal lesions for triphasic CT assessment.
- L5 tiny lytic lesion.
- For correlation with the unavailable previous studies.

Dr. Mohamed Mohsen, M.D.



Patient Name	الأستاذة / عفاف احمد السيد حسوي	Request No.	13003628
Patient No.	272082/0102469	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	28/01/2025
Sex	Female	Financial Category	اورام نادر السلام - نفقة التولية
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Haba Hassan

Verified by :
Dr.Toka Medhat

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Page 2 of 2

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غرفة رقم (4) - الطور الأول - مباني رقم (917) - اورشليم الشرق - مصر الجديدة - القاهرة 1 - مصر
Room No. (4) - First Floor-Building No. 917- Nile Corniche - NMR III Qadima - Cairo, Egypt



Patient Name	: الأستاذة / طيف احمد السيد عسوي	Request No.	: 13003629
Patient No.	: 27206280102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: برنامج ممر السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5iP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Page 1 of 2

Printed By : Mrs.Mayada Hegazy



مركز (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mizr El Qadima - Cairo, Egypt



Patient Name	: الأستاذ / عفاف احمد السيد عيسى	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: فورم دو السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labeled by (P012-25) and one H&I: slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angiolinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature

Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

Printing Date : 22/01/2025 08:14:23

Page 1 of 1

Printing By : internal



شقة رقم (4) - الدور الأول - مبنى رقم (387) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 387- Nile Corniche - Miss El Qadima - Cairo, Egypt



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البيانات الشخصية
بطاقة

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C. 5011A
Age	52	ID	52. 274. 0. 527
Menopausal status		Phone	01 58150472
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NPV

Clinical Data

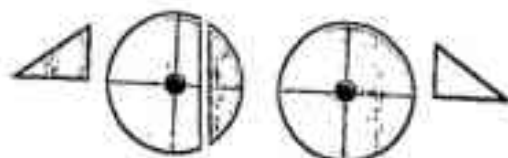
BX: ID 4

ER: 0 PR: 0 HER 2: +3 KI67:

Sonomammography:

Mets Workup:

R



TNM
- Mediastinal L-N
- Bilobar F. L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (paclitaxel) 30 (30)
Then Re-evaluate.

- baseline CT chest, PA etc.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد العبد		د. محمد			

Name	كفان محمد سعيد محمد	MRN	
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC CT			R	L
ER:	PR:	HER 2:	KITV:		
0	0	+3			
sonomammography:					
Mets Workup:				TNM	
Metastatic to breast Ca to bone, liver, L. No Keelated Taxol / Herceptin x 20 weeks CT. CAP 21/5/20 → PD of multiple lesion					

Recommendations:

For MDT for next plan of management

PD of axillary L.N.

Signature: [Signature] 21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : Axillary u/s	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- > Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up
- > Normal skin thickness and subcutaneous tissues
- > Normal vascular structure
- > Signature Dr . Nermien Ramadan
- > 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR Wt's
- Sagittal T1 W1
- Coronal T2 W1
- Axial D'WI and ADC
- Post contrast T1 series

MR findings:

- W dened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- No cerebral, cerebellar or brain stem areas of altered MR signal.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- Mild age matched brain involutional changes.

Dr.Zeyad El-Sayed,FEBR

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

➤ **Stationary course** of the:

1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.

➤ **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.

➤ Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.

➤ No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.

➤ **Stationary course** of the enlarged mediastinal lymph nodes being subcm.

➤ No evidence of pleural effusion, pleural thickening or masses seen.

➤ Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

➤ Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

➤ The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non-contrast study.

➤ No intra-hepatic or extra-hepatic biliary radicles dilatation.

➤ Normal size & parenchymal texture of the spleen with no focal lesions seen.

➤ Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

➤ Normal appearance of the pancreas and both suprarenals.

➤ Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

➤ Normal appearance of the other pelvic organs.

➤ No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

Date 22

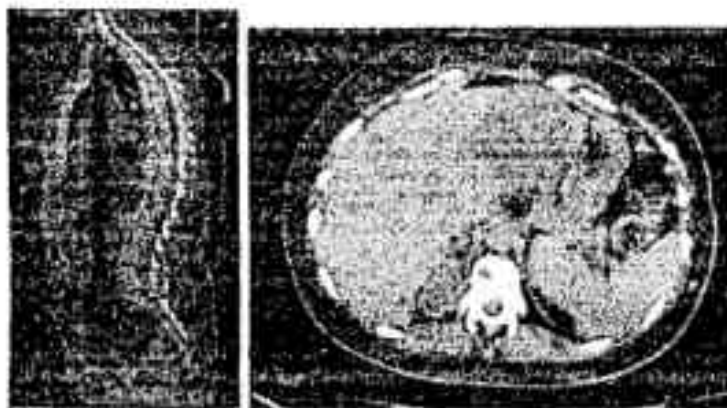
Dr. AYA GABR

Name : Afaf El Sayed

Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texture still showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentimetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.



Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
 1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
 2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847
1720	
1000	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST **PRIMARY REPORT**

- *Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:*



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted subcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afa ² Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1723 1000	Pat. ID: 96847

OPINION:

- *Stationary course of the disease process.*

Marina Essam, MSc



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السيد/الفضيلة
مختبة
مطر

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C 50, 1. A
Age	52	ID	22. 672. 1. 92
Menopausal status		Phone	01 5816472
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	new

BX:

Clinical Data

ER:

PR:

HER 2:

KI67:

Sonomammography:

Mets Workup:

Recommendations:

R

L

TNM Mediastinal L-N
- B. Lpar F. L

MDT OUTCOME & PLAN

1/2/2023 → for 1st line Taxol (herceptin) (IV) (SC)
then Leucovorin

- baseline CT chest, PA etc etc

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

WPC DSHS-MDT 001
REVISED
MAY 10 2
Issue date: 1/9/2022

MDT FORM

/ / 20

G

Name	سافانة احمد	MRN	1081825
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC G1				R	L
ER: 0/8	PR: 0/8	HER 2: +3	KI67:		
Sonomammography:					
Mets Workup: Met breast cancer to liver & LN received 1st line treat weekly + Herceptin last CTCAP → SD. (Gy and to baseline)					
Recommendations: for MDT. i.e. AP Gy and to baseline 5-2025 Consider 2nd line Xeloda-Tx Met 31/7/2025					

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name : الأستاذ / عفاف احمد السيد عيسوي	Request No. : 13003629
Patient No. : 27206260102469	Request Date : 13/01/2025
Path No. : SR-105/25	Verification Date : 21/01/2025
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil



Verified by :
Dr.Heba Hasan



IEYA SCAN

NAME : Afaf Ahmad

DATE: 8/1/2025

PROF .DR; SaedAlbehery



مركز
هي للأشعة

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- ACR B breast parenchyma
- An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen
- Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification
- No dilated lactiferous ducts
- Normal right skin thickness and contour.
- Normal nipple areola complex.
- Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila

OPINION:

- Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)

Much obliged,
DR .Eman AbdelRahman ,MD

001119598889 - 01005425312 @ 0822167074

بنى سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة

بنى سويف - أمام أجيال للملابس



Patient Name: AFAN AHMED ALDAVEED

Patient ID: 324-154

Gender:

Nuclear Imaging Endorsment

Study Name: BONE SCAN

Date & Time: 2/3/2015

Manufacturer Model: VARIUM



فيجنا لاب

لتحاليل الخلايا والأنسجة والأورام

د.د/ إيمان أحمد عبد المونود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د.د/ عبلة سيد دحمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. د.د/ سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At : 5-1-2025

Code Number: P0012-25

Report issued A : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 N.E. & Ki67).



Eman A. Abd Elmaaged

Abla S. Mahmoud

ش. الإبراهيمية امام معرض شباب مصر - بني سويف

ت/ 01017211026

Pathology Report

Patient name: Vis. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03,2021

Tc^{99m}-M.DP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- No scintigraphic evidence of bone deposits throughout the scanned skeleton.

Mahmoud Saban Abd Al-Alim, MD, EBIM

European Board of Nuclear Medicine

فيديا لاب

لتحاليل الخلايا والأنسجة والأورام

د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. أ.د. سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At: 5-1-2025

Code Number: P0012-25

Report issued A: 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests. Individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis.
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass: Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER2/2 NLU & Ki67).



Eman A. Abd Elmaoqed

Abla S. Mahmoud

ش. الأبراهيمية امام معرض شفايف مصر - بني سويف

ت/ 01017211026

Pathology Report
[Signature Pathology Reports]

Patient name: Afaf Ahmed

Date: 4/1/2025

Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped heterogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mild edematous changes manifested by parenchymal coarsening and skin thickening are noted.
- The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.

*Best regards,
Dr. Esraa Hatem*

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0:	Needs further investigations.
BIRADS 1:	Normal findings.
BIRADS 2:	Benign.
BIRADS 3:	Probably benign.
BIRADS 4:	Probably malignant.
BIRADS 5:	Malignant.
BIRADS 6:	Proven pathologically malignant

Dr. Esraa Hatem
Radiology specialist



Patient Name: Afaf Ahmed Al Sayed El sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes; the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- **Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.**
- **Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.**



Patient Name: Afaf Ahmed Al Sayed E. sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Med cal ID: 108/2025	Pet. ID: 96847

- Bilobar hepatic focal lesions for triphasic CT assessment.
- L5 tiny lytic lesion.
- For correlation with the unavailable previous studies.

Dr. Mohamed Mohsen, M.D.



Patient Name	الأستاذة / عفاف احمد السيد حسوي	Request No.	13003628
Patient No.	272082/0102469	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	28/01/2025
Sex	Female	Financial Category	اورام نادر السلام - نفقة التوالة
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Haba Hassan

Verified by :
Dr.Toka Medhat

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Page 2 of 2

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غرفة رقم (4) - الطور الأول - مباني رقم (917) - اورشليم الشرق - مصر الجديدة - القاهرة 1 - مصر
Room No. (4) - First Floor-Building No. 917- Nile Corniche - N.H. Qadima - Cairo, Egypt



Patient Name	: الأستاذ / طيف احمد السيد عسوي	Request No.	: 13003629
Patient No.	: 27206280102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: برنامج ممر السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5iP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Page 1 of 2

Printed By : Mrs.Mayada Hegazy



مركز (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mian El Qadima - Cairo, Egypt



Patient Name	: الأستاذ / عفاف احمد السيد عيسى	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: فورم دو السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labeled by (P012-25) and one H&I: slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angiolinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature

Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

Printing Date : 22/01/2025 08:14:23

Page 1 of 1

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شقة رقم (4) - الدور الأول - مبنى رقم (387) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 387- Nile Corniche - Miss El Qadima - Cairo, Egypt



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البيانات الشخصية
بطاقة

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C. 50110A
Age	52	ID	52. 5754. 527
Menopausal status		Phone	0158150472
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NPV

Clinical Data

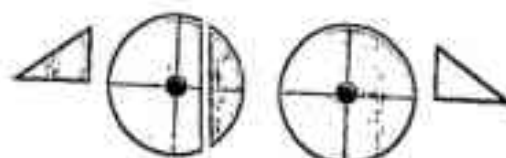
BX: ID 4

ER: 0 PR: 0 HER 2: +3 KI67:

Sonomammography:

Mets Workup:

R



TNM
- Mediastinal L-N
- Bilobar F. L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (paclitaxel) 30 (30)
Then Re-evaluate.

- baseline CT chest, PA etc.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد العبد		د. محمد			

Name	كفان محمد سعيد محمد	MRN	
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC CT			R	L
ER:	PR:	HER 2:	KITV:		
0	0	+3			
sonomammography:					
Mets Workup:				TNM	
Metastatic to breast Ca to bone, liver, L. No Kosideral Taxol / Herceptin x 20 weeks CT. CAP 21/5/20 → PD of multiple lesion					

Recommendations:

For MDT for next plan of management

PD of axillary L.N.

[Signature]
21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Afaf Ahmed Al Sayed Eisawy

Date : 21/07/2025

Exam : Axillary u/s

Age : 53 Year(s)

Medical ID: 108/2025

Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- > Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up
- > Normal skin thickness and subcutaneous tissues
- > Normal vascular structure
- > Signature Dr . Nermien Ramadan
- > 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WIs
- Sagittal T1 W1
- Coronal T2 W1
- Axial D'WI and ADC
- Post contrast T1 series

MR findings:

- Well defined cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- No cerebral, cerebellar or brain stem areas of altered MR signal.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- Mild age matched brain involutional changes.

Dr.Zeyad El-Sayed,FEBR

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

➤ **Stationary course** of the:

1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.

➤ **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.

➤ Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.

➤ No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.

➤ **Stationary course** of the enlarged mediastinal lymph nodes being subcm.

➤ No evidence of pleural effusion, pleural thickening or masses seen.

➤ Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

➤ Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

➤ The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non-contrast study.

➤ No intra-hepatic or extra-hepatic biliary radicles dilatation.

➤ Normal size & parenchymal texture of the spleen with no focal lesions seen.

➤ Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

➤ Normal appearance of the pancreas and both suprarenals.

➤ Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

➤ Normal appearance of the other pelvic organs.

➤ No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

Date 22

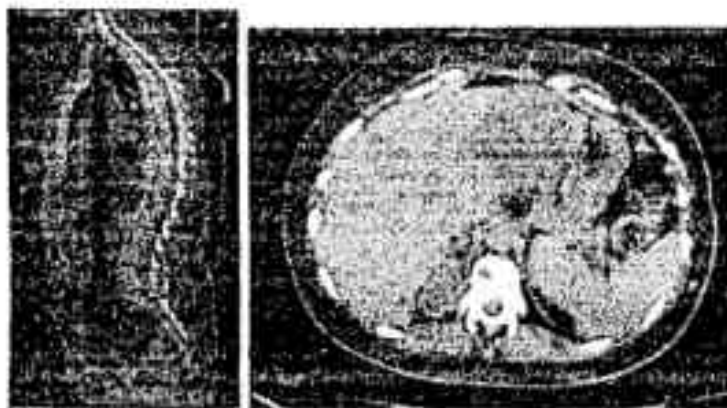
Dr. AYA GABR

Name : Afaf El Sayed

Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texture still showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentimetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.



Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
 1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
 2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847
1720	
1000	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST **PRIMARY REPORT**

- *Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:*



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted subcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afa ² Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1723 1000	Pat. ID: 96847

OPINION:

- *Stationary course of the disease process.*

Marina Essam, MSc



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السيد/الفضيلة
مختبة
مطر

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C 50, 1. A
Age	52	ID	20. 670. 1. 02
Menopausal status		Phone	01 58160475
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	new

BX:

Clinical Data

ER:

PR:

HER 2:

KI67:

Sonomammography:

Mets Workup:

Recommendations:

R

L

TNM Mediastinal L-N
- B. Lpar F. L

MDT OUTCOME & PLAN

1/2/2023 → for 1st line Taxol (herceptin) (IV) (SC)
then Leovante.

- baseline CT chest, PA etc etc.

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

WPC DSHS-MDT 001
REVISED
MAY 10 2
Issue date: 1/9/2022

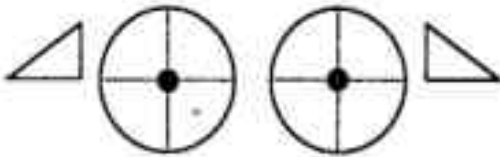
MDT FORM

/ / 20

G

Name	سافانة احمد	MRN	1081825
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC G1	R	L
ER: 0/8	PR: 0/8	HER 2: +3
Sonomammography:		
		
Mets Workup: Mets breast cancer to liver & LN received 1st line treatment weekly + Herceptin last CTCAP → SD. (Gy and to baseline)		
Recommendations: for MDT. 1.e. AP Gy and to baseline 5-2025 Consider 2nd line Xeloda-Tx Met		

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name : الأستاذ / عفاف احمد السيد عيسوي	Request No. : 13003629
Patient No. : 27206260102469	Request Date : 13/01/2025
Path No. : SR-105/25	Verification Date : 21/01/2025
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil



Verified by :
Dr.Heba Hasan



IEYA SCAN

NAME : Afaf Ahmad

DATE: 8/1/2025

PROF .DR; SaedAlbehery



مركز
هي للأشعة

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- ACR B breast parenchyma
- An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen
- Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification
- No dilated lactiferous ducts
- Normal right skin thickness and contour.
- Normal nipple areola complex.
- Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila

OPINION:

- Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)

Much obliged,
DR .Eman AbdelRahman ,MD

001119598889 - 01005425312 @ 0822167074

بنى سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة

بنى سويف - أمام أجيال للملابس



Patient Name: AFAN AHMED ALDAVEED

Patient ID: 324-154

Gender:

Nuclear Imaging Endorsment

Study Name: BONE SCAN

Date & Time: 2/3/2015

Manufacturer Model: VARIUM



فيجنا لاب

لتحاليل الخلايا والأنسجة والأورام

د.د/ إيمان أحمد عبد المونود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

د.د/ عبلة سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كلية الطب
البشري

Patient's
Data

Patient Name: عفاف أحمد السيد

Age: 51 y

Gender: Female

Referred By Prof. Dr. د.د/ سعيد البحري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At : 5-1-2025

Code Number: P0012-25

Report issued A : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 N.E. & Ki67).



Eman A. Abd Elmaaged

Abla S. Mahmoud

ش. الإبراهيمية امام معرض شباب مصر - بني سويف

ت/ 01017211026

Pathology Report

Patient name: Vis. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03,2021

Tc^{99m}-M.DP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- No scintigraphic evidence of bone deposits throughout the scanned skeleton.

Mahmoud Saban Abd Al-Alim, MD, EBIM

European Board of Nuclear Medicine

Patient name: Afaf Ahmed

Date: 4/1/2025

Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped heterogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mild edematous changes manifested by parenchymal coarsening and skin thickening are noted.
- The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.

*Best regards,
Dr. Esraa Hatem*

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0:	Needs further investigations.
BIRADS 1:	Normal findings.
BIRADS 2:	Benign.
BIRADS 3:	Probably benign.
BIRADS 4:	Probably malignant.
BIRADS 5:	Malignant.
BIRADS 6:	Proven pathologically malignant

Dr. Esraa Hatem
Radiology specialist



Patient Name: Afaf Ahmed Al Sayed El sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes; the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.
- Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.



Patient Name: Afaf Ahmed Al Sayed E. sawy	Date : 13/01/2025
Exam : C.T ABDOMEN & PELVIS	Age : 52 Year(s)
Med cal ID: 108/2025	Pet. ID: 96847

- Bilobar hepatic focal lesions for triphasic CT assessment.
- L5 tiny lytic lesion.
- For correlation with the unavailable previous studies.

Dr. Mohamed Mohsen, M.D.



Patient Name	الأستاذة / عفاف احمد السيد حسوي	Request No.	13003628
Patient No.	272082/0102469	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	28/01/2025
Sex	Female	Financial Category	اورام نادر السلام - نفقة التولية
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Haba Hassan

Verified by :
Dr.Toka Medhat

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 917- Nile Corniche - N.H. Qadima - Cairo, Egypt



Patient Name	: الأستاذة / طيف احمد السيد عسوي	Request No.	: 13003629
Patient No.	: 27206280102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: برنامج ممر السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5iP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Page 1 of 2

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مركز (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mizr El Qadima - Cairo, Egypt



Patient Name	: الأستاذ / عفاف احمد السيد عيسى	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: فورم دو السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labeled by (P012-25) and one H&I: slide were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * DCIS: Focally positive.
- * Angiolinvasion: Could not be detected within the limits of biopsy received

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature

Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

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Page 1 of 1

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شقة رقم (4) - الدور الأول - مبنى رقم (387) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البيانات الشخصية
بطاقة

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C. 5011A
Age	52	ID	52. 574. 527
Menopausal status		Phone	0158150472
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NPV

Clinical Data

BX: ID 4

ER: 0 PR: 0 HER 2: +3 KI67:

Sonomammography:

Mets Workup:

R L
TNM
- Mediastinal L.N
- Bilobar F. L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (paclitaxel) 30 (30)
Then Re-evaluate.

- baseline CT chest, PA etc.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد العبد		د. محمد			

Name	كفان محمد سعيد محمد	MRN	
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC CT			R	L
ER:	PR:	HER 2:	KITV:		
0	0	+3			
sonomammography:					
Mets Workup:				TNM	
Metastatic to breast Ca to bone, liver, L. No Keelated Taxol / Herceptin x 20 weeks CT. CAP 21/5/20 → PD of multiple lesion					

Recommendations:

For MDT for next plan of management

PD of axillary L.N.

[Signature]
21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Afaf Ahmed Al Sayed Eisawy

Date : 21/07/2025

Exam : Axillary u/s

Age : 53 Year(s)

Medical ID: 108/2025

Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- > Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up
- > Normal skin thickness and subcutaneous tissues
- > Normal vascular structure
- > Signature Dr . Nermien Ramadan
- > 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR Wt's
- Sagittal T1 W1
- Coronal T2 W1
- Axial D'WI and ADC
- Post contrast T1 series

MR findings:

- W dened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- No cerebral, cerebellar or brain stem areas of altered MR signal.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- Mild age matched brain involutional changes.

Dr.Zeyad El-Sayed,FEBR

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

➤ **Stationary course** of the:

1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.

➤ **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.

➤ Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.

➤ No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.

➤ **Stationary course** of the enlarged mediastinal lymph nodes being subcm.

➤ No evidence of pleural effusion, pleural thickening or masses seen.

➤ Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

➤ Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

➤ The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non-contrast study.

➤ No intra-hepatic or extra-hepatic biliary radicles dilatation.

➤ Normal size & parenchymal texture of the spleen with no focal lesions seen.

➤ Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

➤ Normal appearance of the pancreas and both suprarenals.

➤ Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

➤ Normal appearance of the other pelvic organs.

➤ No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

Date 22

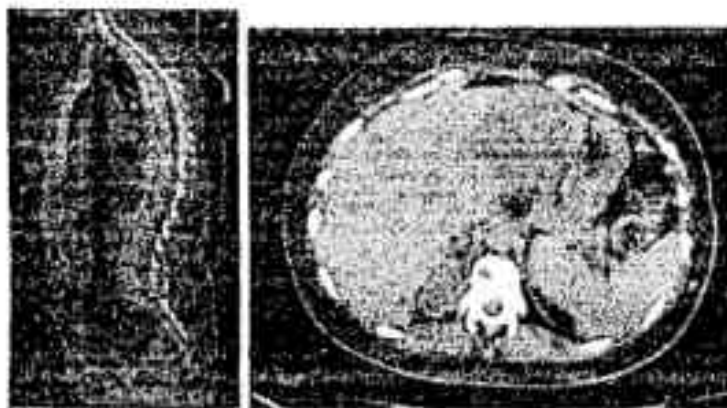
Dr. AYA GABR

Name : Afaf El Sayed

Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texture still showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentimetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.



Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
 1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
 2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847
1720	
1000	

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST **PRIMARY REPORT**

- *Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:*



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted subcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afa ² Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1723 1000	Pat. ID: 96847

OPINION:

- *Stationary course of the disease process.*

Marina Essam, MSc



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السيد/المنصبة
مختة
مطر

MDT FORM

1 / 2 / 20 25 GM

Name	عفاف أحمد السيد عيسى	MRN	C 50, 1. A
Age	52	ID	22. 672. 1. 92
Menopausal status		Phone	01 58165475
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	new

BX:

Clinical Data

ER:

PR:

HER 2:

KI67:

C

0

+3

Sonomammography:

Mets Workup:

R

L

TNM Mediastinal L-N
- B. Lpar F. L

Recommendations:

MDT OUTCOME & PLAN

1/2/2023 → for 1st line Taxol (herceptin) (IV) (SC)
then Leucovorin

- baseline CT chest, PA etc etc

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

WPC DSHS-MDT-001
REVISED
MAY 10 2
Issue date: 1/9/2022

MDT FORM

/ / 20

G

Name	سافانة احمد	MRN	1081825
Age	52	ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC G1	R	L
ER: 0/8	PR: 0/8	HER 2: +3
Sonomammography:		
Mets Workup: Mets breast cancer to liver & LN received 1st line treat weekly + Herceptin last CTCAP → SD. (Gy and to baseline)		
Recommendations: for MDT. 1-e. AP Gy and to baseline 5-2025 Consider 2nd line Xeloda-Tx Met		

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric